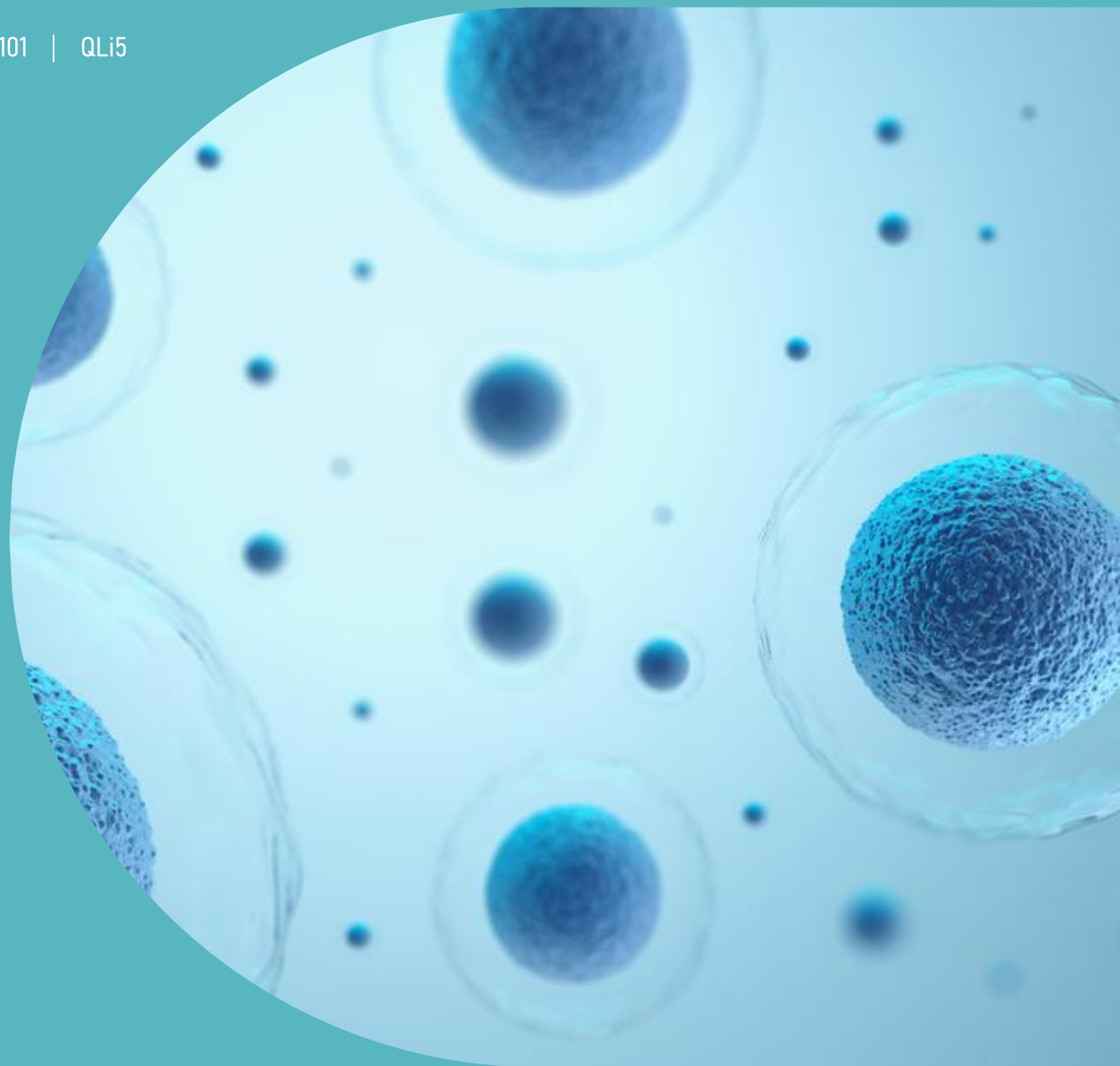




Mocaciclib(Q901) | Adrixetinib(Q702) | Telacebec(Q203) | QP101 | QLi5

Investor Relations 2026

Innovation for Unmet Medical Needs





Disclaimer

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The forecast information contained in this material is related to future events and inherently contains uncertainties.

Please note that the results of the actual business performance of the company may not match due to uncertainties such as fluctuations in the market environment and risks beyond the control of the company.

Finally, this data is prepared as a reference for investors to make investment decisions.

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Quriént internet homepage : <http://www.quriént.com/>



Labless Research House

Building high-value therapeutics through scalable science

Established with support from government and international institutions to advance innovative drug development

Validated development capabilities through advancement of high-potential pipelines

Entering a value realization phase through capital reinforcement, milestone execution, and licensing outcomes



Establishment

- Jul 2008 Spun off from Institut Pasteur Korea (IPK)
- Mar 2010 In-licensed Telacebec (Q203) from IPK
- Mar 2013 In-licensed Adrixetinib (Q702) from Max Planck Institute (MPI) / LDC
- Feb 2015 In-licensed Mocaciclib (Q901) from MPI / LDC



Development

- Jul 2015 Telacebec (Q203) entered clinical development
- Feb 2016 Listed on KOSDAQ
- Jan 2020 Established subsidiary QLi5 Therapeutics
- May 2020 Adrixetinib (Q702) entered clinical development
- Nov 2021 Entered into a co-development agreement with MSD for Adrixetinib (Q702)
- Sep 2022 Initiated Q702 + Keytruda combination trial



Achievement

- Feb 2023 Licensed Telacebec to TB Alliance
- Jun 2023 Mocaciclib (Q901) entered clinical development
- Aug 2023 Entered into a collaborative R&D agreement with the U.S. National Cancer Institute (NCI) for Mocaciclib (Q901)
- Apr 2024 Strategic investment from DongKoo Bio & Pharma, strengthening commercialization capabilities
- May 2024 Initiated Telacebec registrational study in Australia
- Sep 2025 In-licensed technology from Synaffix B.V. for development of dual-payload ADCs



Our business model

“Labless Research House” building high-value therapeutics through scalable science



“Qurient concentrates resources on science, decision-making, and value creation — not fixed infrastructure.”

- Scientific strategy & biology
- Program ownership (IP)
- Clinical development leadership

Global execution network



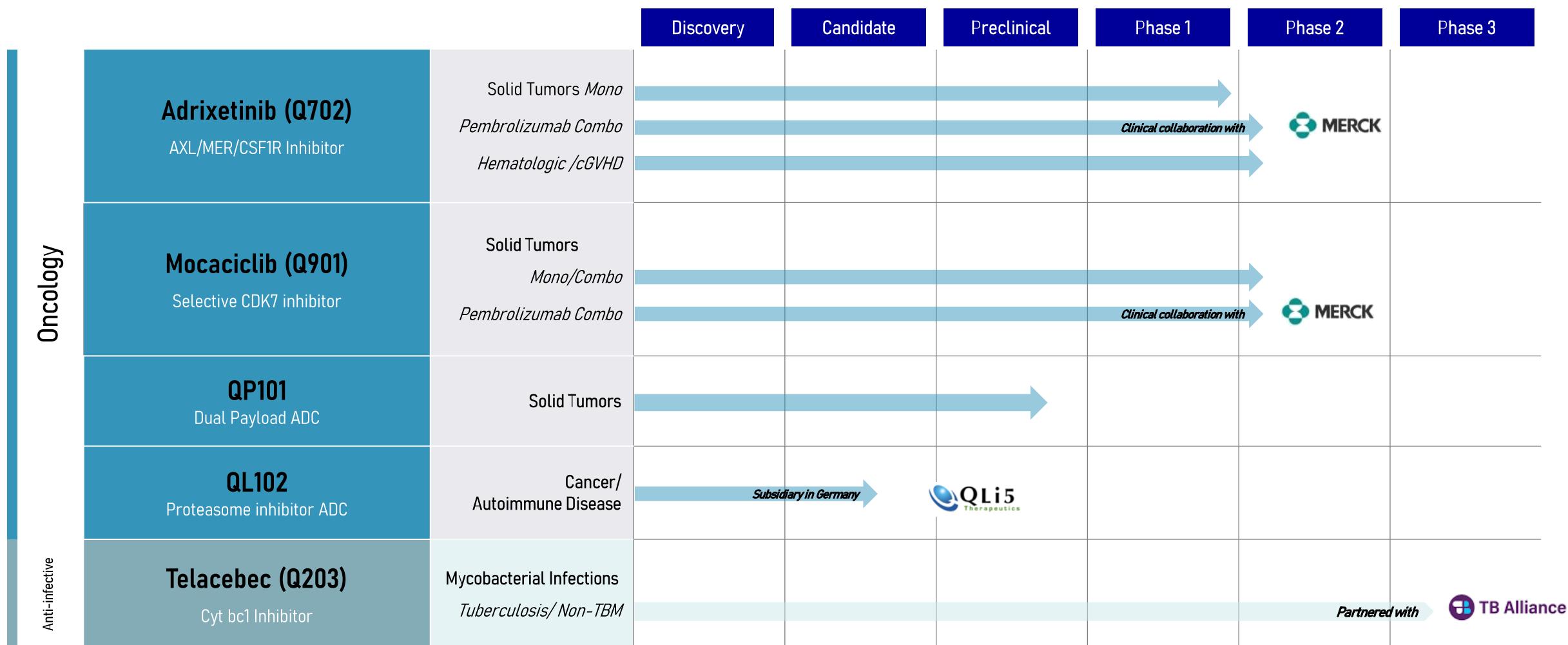
- CROs and CMOs
- Specialized research laboratories
- Academic, clinical development collaborations



High-value therapeutic assets

- Oncology and infectious disease programs
- Clinical-stage and BD-ready assets

Key pipeline progress

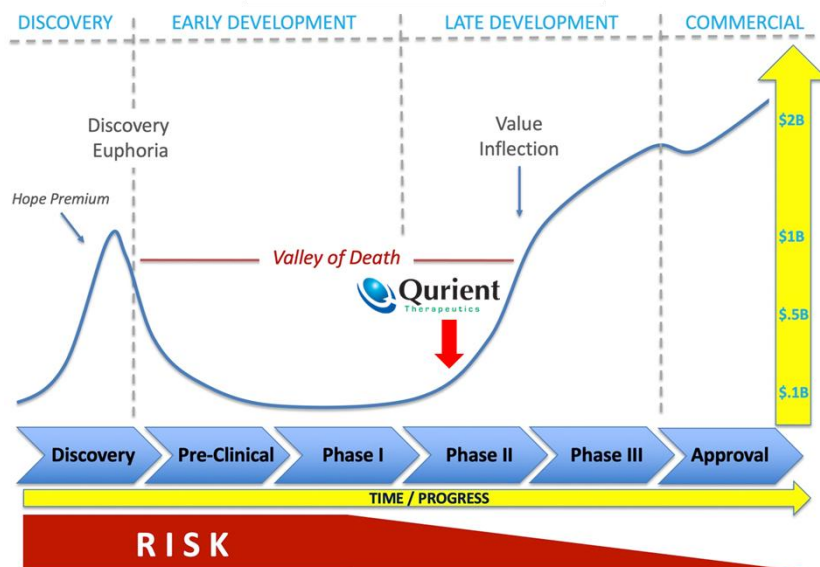




Entering the Value Inflection Phase

Key oncology programs are entering clinical proof-of-concept and commercialization stages

Biotech Value Inflection Cycle



Positioned at the Center of Key Global Oncology Trends

Mono + ADC Synergy

Mocaciclib (Q901)

Dual-Payload ADC

QP Series ADC (QP101)

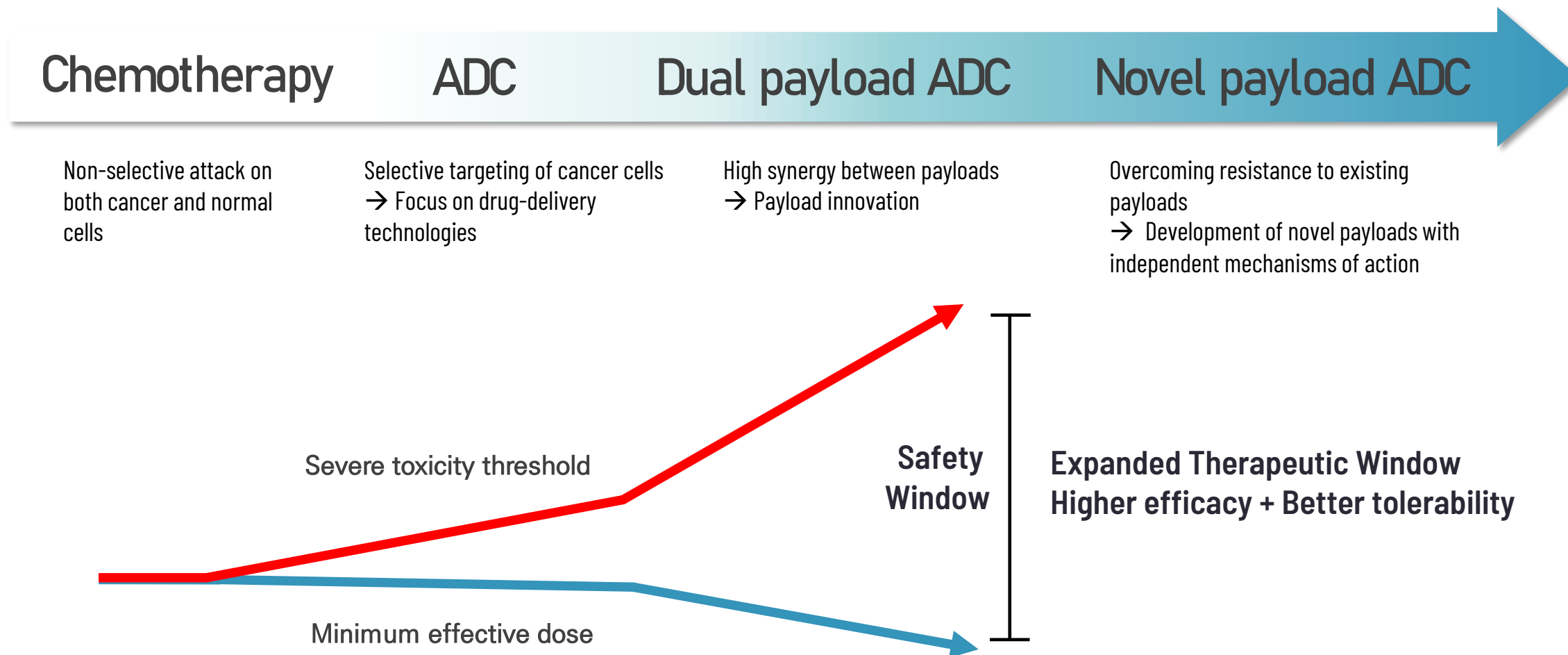
Novel ADC Payload

Proteasome Inhibitor
(QLi5)

cGvHD + Blood Cancer

Adrixetinib (Q702)

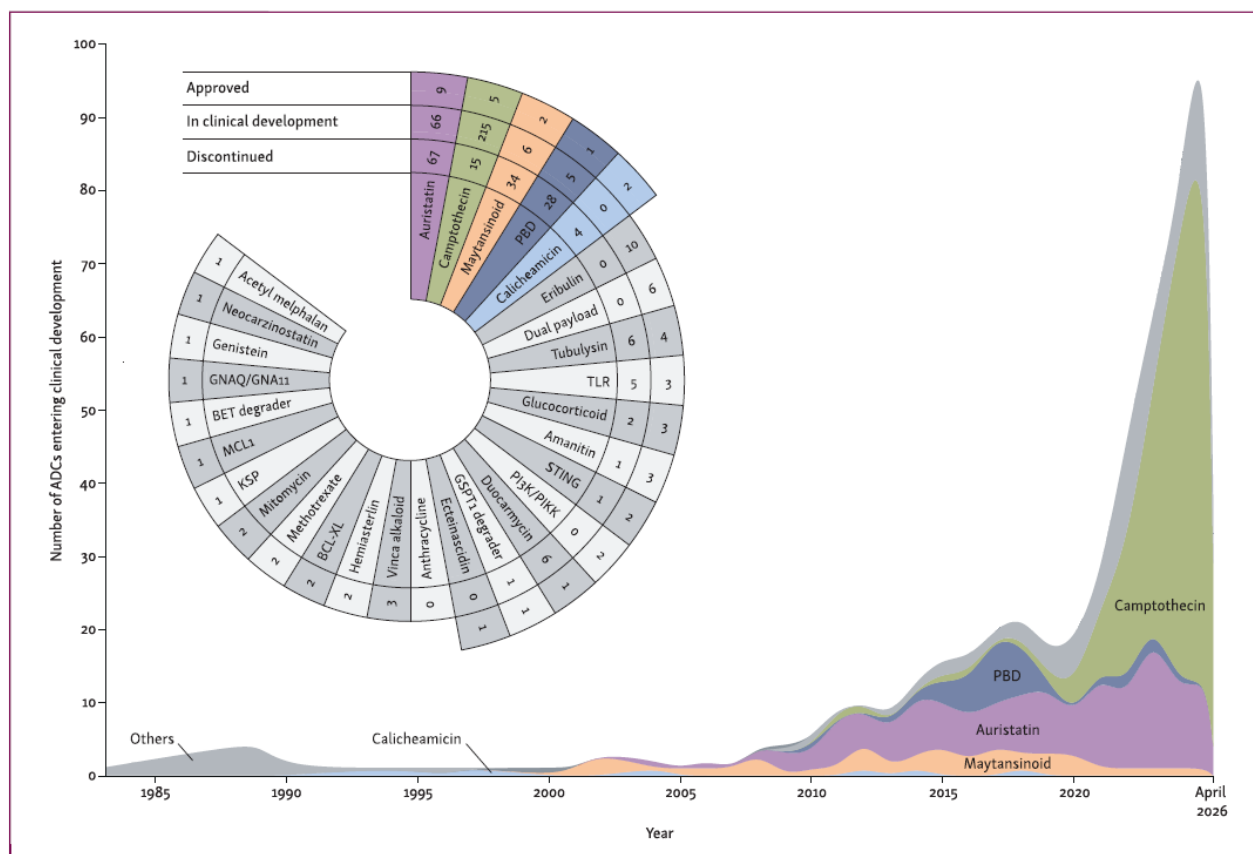
Oncology Innovation Shifting: ADCs → Dual-Payload ADCs



Most of ADC in development uses TOP1 inhibitors

Annals of Oncology

R. Colombo et al.



Topoisomerase 1 Inhibitor (TOP1i)

Figure 5. Current ADC landscape as of March 2026. More than 300 ADCs are in clinical development. Payload class is undisclosed for 17 ADCs. The landscape encompasses 557 ADCs that have entered the clinic, 19 approved ADCs (14 approved by US Food and Drug Administration with additional 5 approved only by China NMPA), 342 ADCs in clinical development and 196 discontinued ADCs. The figure illustrates a rapid expansion of camptothecin-based ADCs over the past 5 years, alongside the emergence of new payload classes and dual-payload ADCs. Updated from reference.³ ADC, antibody–drug conjugate; NMPA, National Medical Products Administration.

(R. Colombo et al., Annals of Oncology, 2026;14:1–20)

Intense ADC Competition: The Need for +α Boosters

Intensifying competition creates a growing need for synergistic agents that enhance ADC efficacy and differentiation

TROP2 ADCs in Later-Line (≥2L) mTNBC

Drug	Key Clinical Trial	Target Population	Objective Response Rate (ORR)	Median Progression-Free Survival (mPFS)	Median Overall Survival (mOS)
Sacituzumab govitecan (Trodelvy)	ASCENT (Phase 3)	≥2nd-line mTNBC	31.0% vs 4.0% (ICC)	5.6 mo vs 1.7 mo (HR: 0.41)	12.1 mo vs 6.7 mo (HR: 0.51)
Sacituzumab tirumotecan (sac-TMT / SKB264)	OptiTROP-Breast01 (Phase 3)	≥2nd-line mTNBC	43.8% vs 12.8% (ICC)	5.7 mo vs 2.3 mo (HR: 0.31)	Not Reached vs 9.4 mo (HR: 0.53)
Datopotamab deruxtecan (Datroway / Dato-DXd)	TROPION-PanTumor01 (Phase 1)	Heavily pretreated mTNBC	31.8% (Single-arm)	Phase 3 data pending for ≥ 2L TNBC	Phase 3 data pending for ≥ 2L TNBC

(Trodelvy, *Journal of Clinical Oncology* 2024;42(15):1738–1744)

(sac-TMT, *Nature Medicine* 2025;31(6):1969–1975)

(Dato-DXd, *Journal of Clinical Oncology* 2024;42(19):2281–2294)

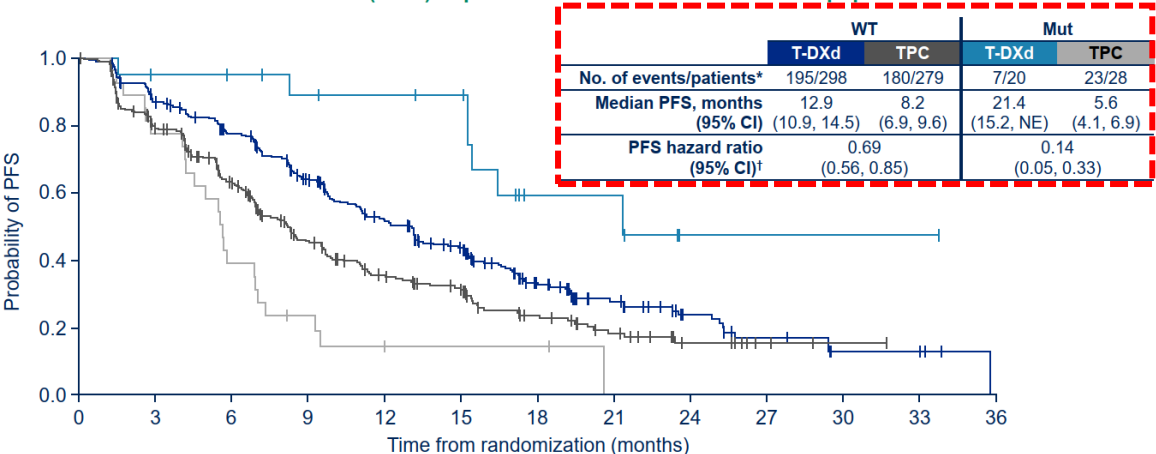
What Is the Key to Improving ADC Efficacy?

Clinical data suggest enhanced ADC efficacy in HRD tumors → HRD induction may be a key strategy

Homologous recombination deficiency (HRD) is a state in which cells are impaired in repairing DNA double-strand breaks through the homologous recombination repair (HRR) pathway, often involving BRCA1/2 dysfunction.

PFS (BICR) by baseline BRCA1/2 mutation status

BRCA1/2 mutations were observed in 7.7% (n=48) of patients in the biomarker-evaluable population



DESTINY-Breast06

T-DXd improved PFS versus TPC regardless of BRCA1/2 mutation status; a potentially greater benefit with T-DXd was observed in the Mut subgroup (albeit with a small sample size)

Vertical lines indicate censored observations; CIs for median were derived based on Brookmeyer-Crowley method; BICR according to RECIST 1.1. *Number of patients with mutation; †hazard ratios and CIs were based on a Cox proportional hazards model with no stratification factors, and ties handled by Efron approach (a hazard ratio <1 favored T-DXd vs TPC). BICR, blinded independent central review; CI, confidence interval; Mut, mutation; NE, not evaluable; PFS, progression-free survival; RECIST, Response Evaluation Criteria in Solid Tumours; T-DXd, trastuzumab deruxtecan; TPC, physician's choice of chemotherapy; WT, wild type

PRESENTED BY: Rebecca Dent, MD, MSC

(Enhertu, 2025 ASCO; *Journal of Clinical Oncology*, 43(16_suppl), 1013–1013)

ASCENT-03

Subgroup	SG + Pembro		Chemo + Pembro	
	n	mPFS, mo	n	mPFS, mo
ITT Population ^{1,a}	221	11.2 (9.3-16.7)	222	7.8 (7.3-9.3)
tBRCA BAS ^b	169	11.1 (9.2-16.7)	163	7.8 (7.2-9.3)
tBRCA subgroup ^b				
WT	130	9.6 (7.6-16.7)	131	7.4 (6.9-9.2)
Mut	39	16.6 (7.5-NE)	32	12.9 (7.1-NE)

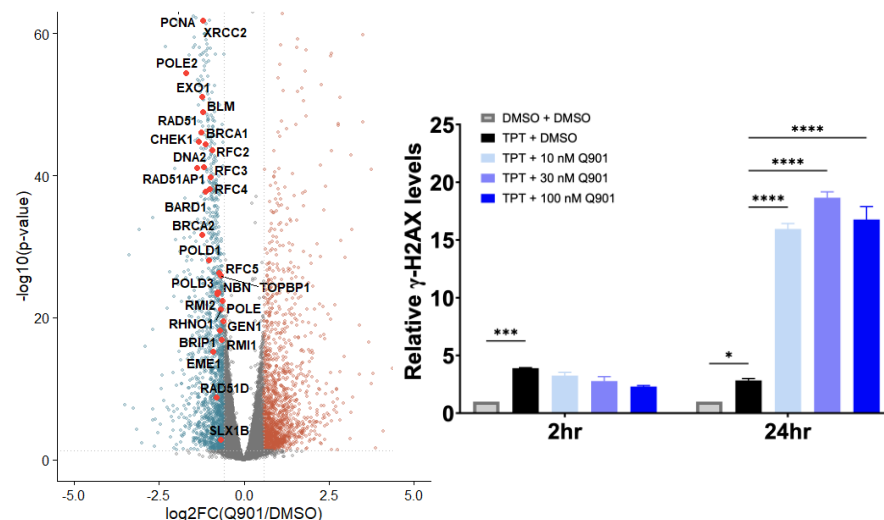
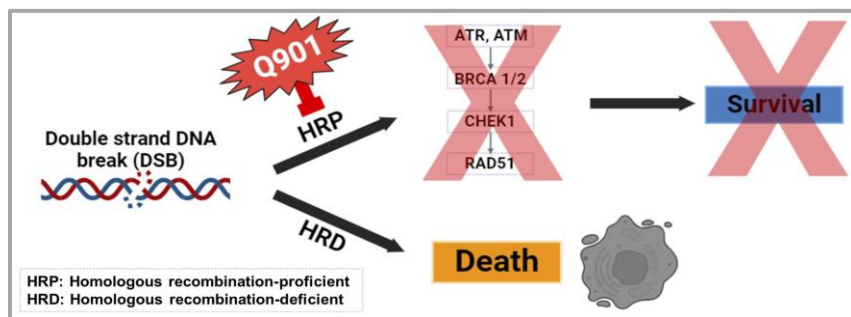
Subgroup	SG		Chemo	
	n	mPFS, mo	n	mPFS, mo
ITT Population ^{1,a}	279	9.7 (8.1-11.1)	279	6.9 (5.6-8.2)
tBRCA BAS ^b	217	9.6 (8.0-11.1)	206	7.2 (5.6-8.3)
tBRCA subgroup ^b				
WT	177	8.8 (7.2-9.9)	169	6.9 (5.4-8.3)
Mut	40	12.7 (7.2-18.7)	37	8.3 (5.6-11.2)

ASCENT-04

(Trodelvy, 2026 ASCO; *Journal of Clinical Oncology*, 44(16_suppl), 1013–1014)

Q901: A Potent HRD-Inducing +a Asset

AACR 2024 and a 2025 publication demonstrated Q901-induced HRD and strong synergy with Enhertu

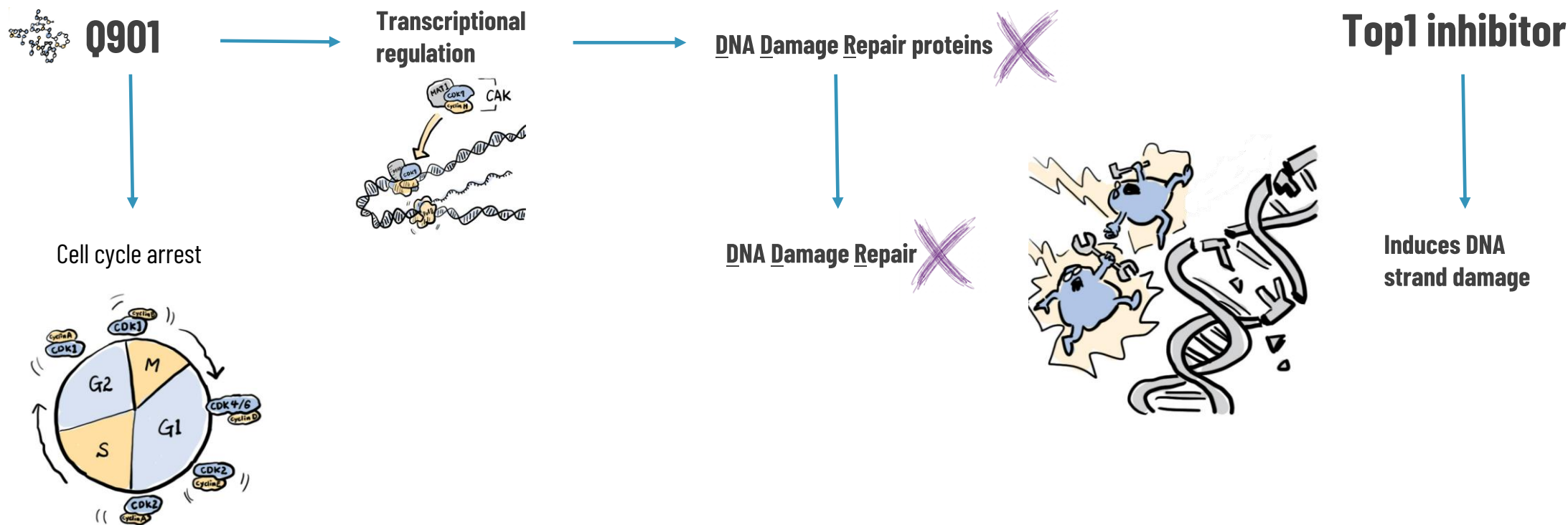


- ✓ Q901 reduces transcription of HRD-related genes
- ✓ A novel mechanism was demonstrated whereby Q901 markedly enhances sensitivity to TOP1i-ADCs such as Enhertu
- ✓ Q901 induces HRD through transcriptional regulation

➔ **A Clear differentiation strategy through Q901 combination therapy**

Q901 – A Breakthrough Transforming the ADC Landscape

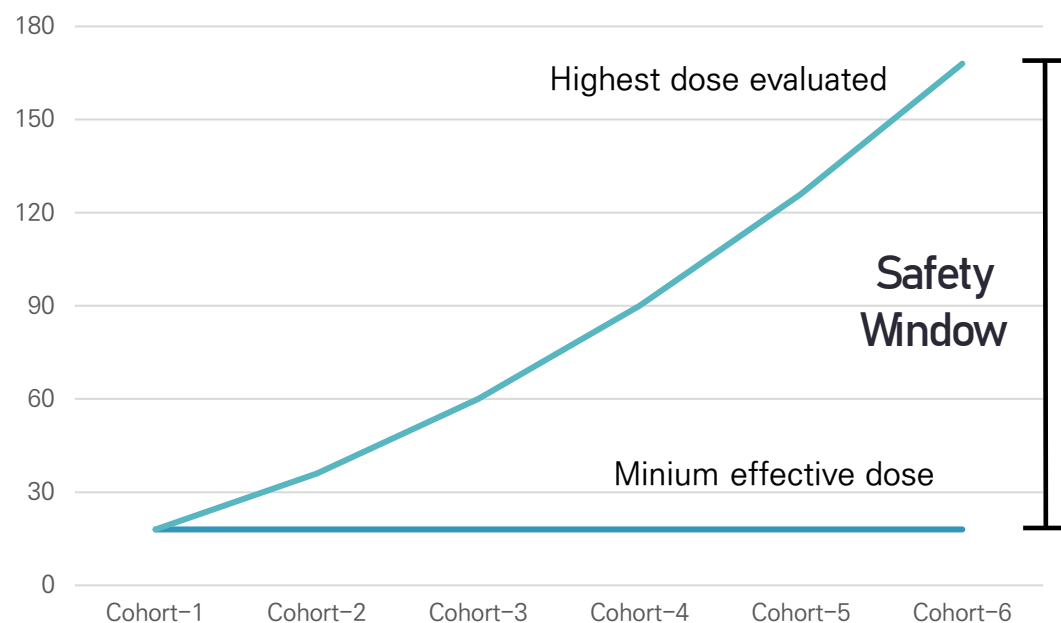
CDK7 inhibition delivers potent anticancer activity by modulating the cell cycle and RNA transcription



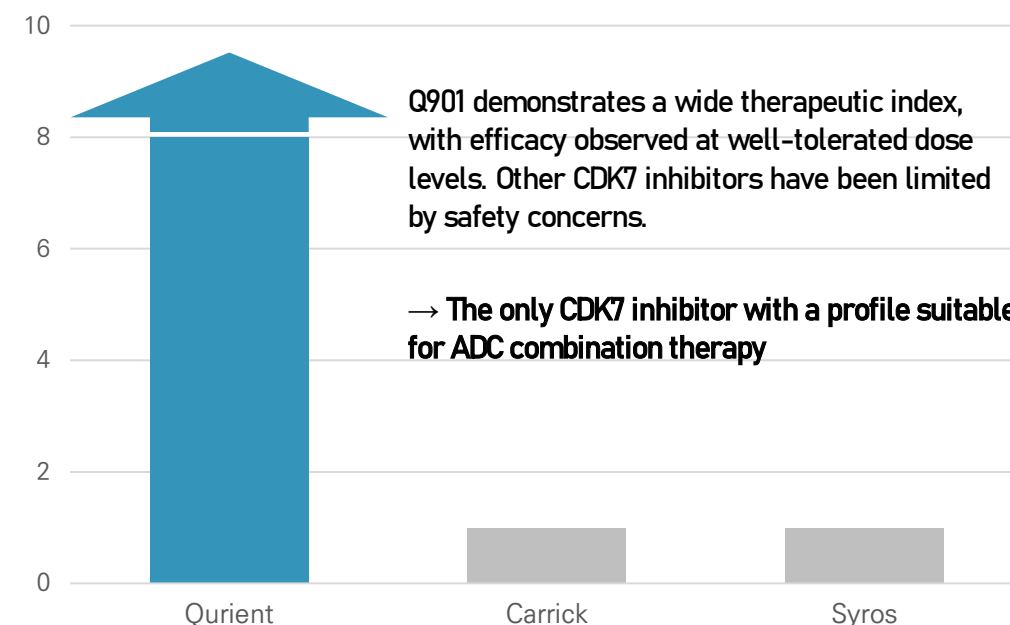
Q901 – Established Safety Profile for ADC Combination Therapy

Clinical safety profile supported by RP2D (Recommended Phase 2 Dose) determination in January 2026

Q901 Dose Escalation Study



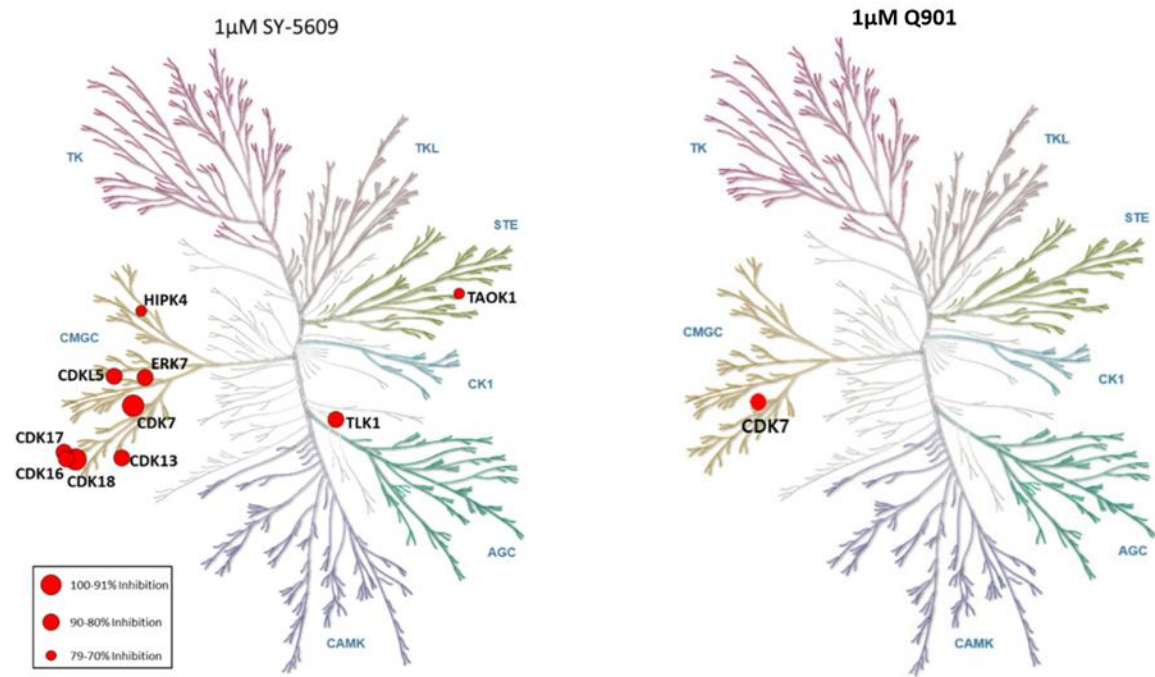
Therapeutic Index



Q901's High Selectivity → Safety

Extremely high selectivity and favorable drug-like properties of Q901

Kinase Selectivity: Syros SY-5609 vs. Qurient Q901



Q901 selectively inhibits CDK7 across a 510-kinase panel

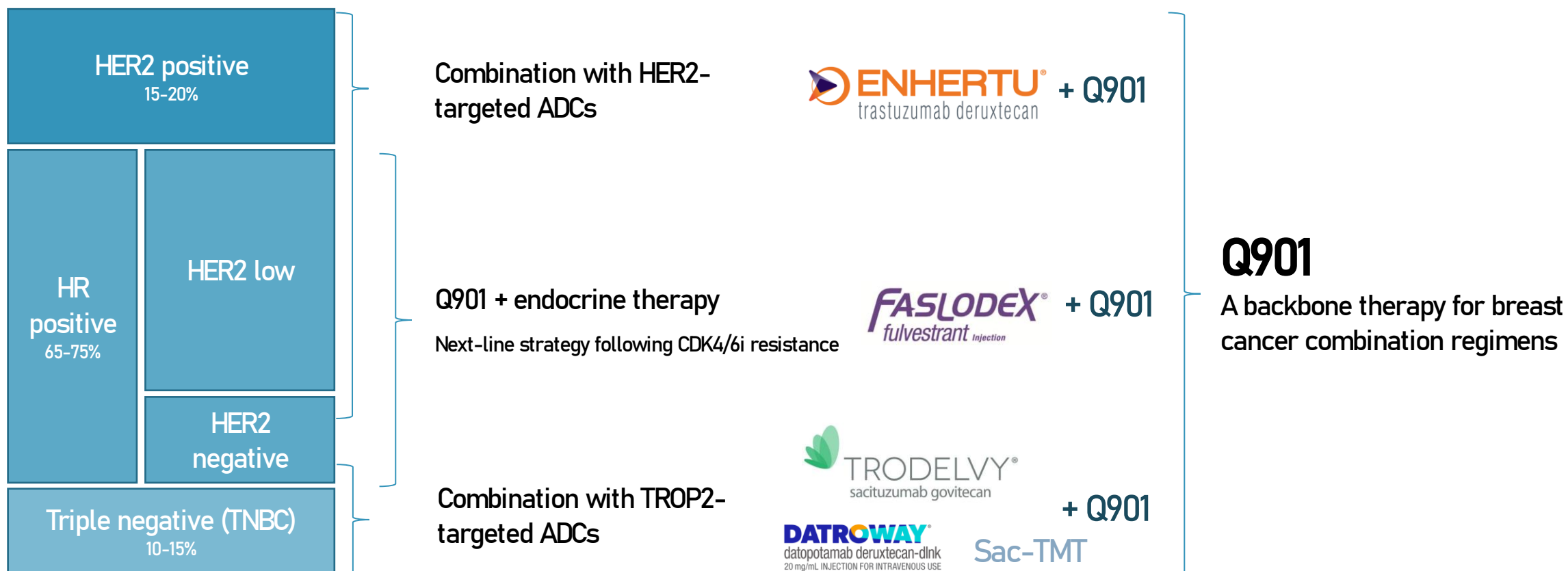
Kinase	Q901 IC ₅₀ (µM)
CDK7/CycH/MAT1	0.012
CDK1/CycA2	8.71
CDK1/CycB1	>10
CDK1/CycE1	5.00
CDK12wt/CycK	2.48
CDK13/CycK	7.69
CDK16/CycY	6.94
CDK17/p35NCK	>10
CDK19/CycC	>10
CDK2/CycA2	>10
CDK2/CycD1	>10
CDK2/CycE1	8.09
CDK20/CycH	>10
CDK20/CycT1	>10
CDK3/CycC	>10
CDK3/CycE1	>10
CDK4/CycD1	>10
CDK4/CycD2	>10
CDK4/CycD3	>10
CDK5/p25NCK	>10
CDK5/p35NCK	>10
CDK6/CycD1	>10
CDK6/CycD2	>10
CDK6/CycD3	>10
CDK8/CycC	>10
CDK9/CycK	8.58
CDK9/CycT1	>10

Leading CDK7 Inhibitor with Proven Competitive Advantages

Candidate	Mocaciclib (Q901)	Samuraciclib (CT7001)	REC-617	SY5609	XL102
Company	Qurient	Carrick Therapeutics	Recursion Pharma	Syros Pharma	Exelixis
Development Status	Phase 2 Co-development with Merck Co-development with NCI (SCLC) Combination with SERD Combination with TROP2 ADC	Phase 2 Pfizer investment Combination development with multiple hormone therapies and endocrine agents	Phase 1	Development discontinued	Development discontinued
Administration Route	Intravenous injection	Oral	Oral	Oral	Oral
Selectivity (# of off-target CDK)	0	8	2	2	3
Pharmacodynamic Analysis	PD marker changes observed from 18 mg	PD marker changes observed from 240 mg	PD marker changes observed from 10 mg	Observed from RP2D	70% target inhibition confirmed at maximum dose
Recommended Phase 2 Dose (RP2D)	126mg	360 mg daily	Not determined (>20 mg QD))	3 mg daily or 10 mg daily, intermittent dosing)	Clinical development discontinued due to toxicity issues
Maximum Tolerated Dose (MTD)	Not determined (>168 mg)	360 mg daily	Not determined (>20 mg QD)	3 mg daily or 10 mg daily, intermittent dosing)	
First Response Dose	18 mg once every 2 weeks	240 mg daily	20 mg daily	None	
Therapeutic index	Data to be presented at ESMO	1~1.5 (360mg QD / 360mg QD)	N/A	1 (4mg QD / 4mg QD) On hold due to toxicity issues	
Safety Profile	Data to be presented at ESMO	Grade 3 diarrhea, nausea/vomiting, anemia, platelet count↓	Grade 3 diarrhea, vomiting, platelet count↓	Grade 3 nausea, diarrhea, vomiting, platelet count↓, fatigue	40mg BID showing Grade 3 AST↑, and Grade 4 ALT↑

Q901 – Breast Cancer Market Expansion Strategy

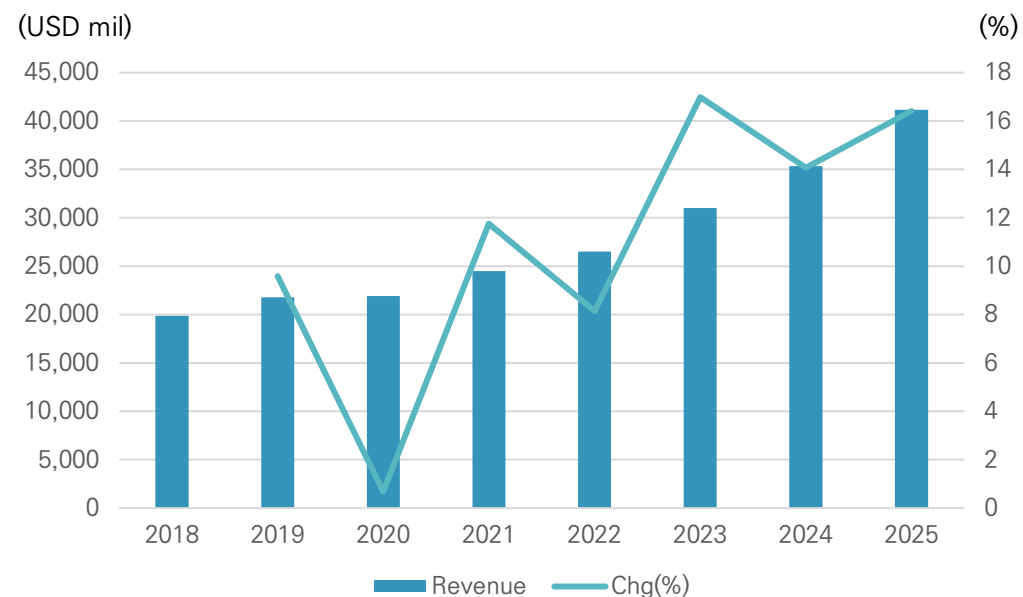
Broad combination potential across major breast cancer subtypes: Divide and Conquer



Q901's Primary Target Market – Breast Cancer

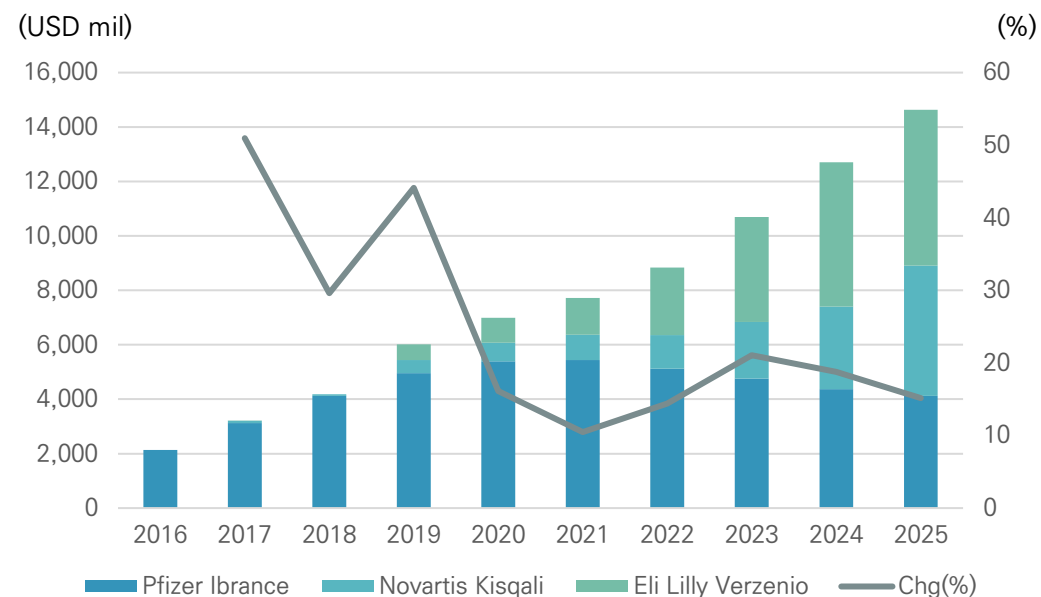
The breast cancer market reached KRW 60t in 2025, with CDK4/6 inhibitors accounting for 36% of the market.

Breast Cancer Therapeutics Market Size



자료: Evaluate Pharma, 각사 실적

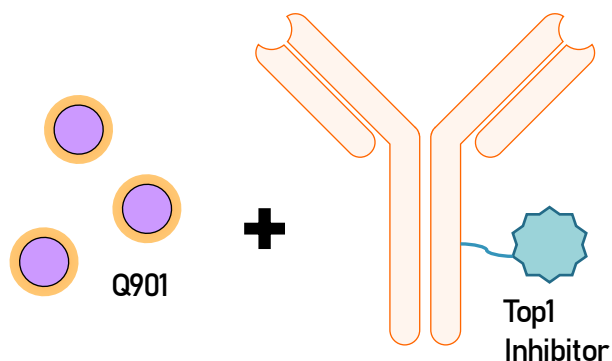
CDK4/6 Inhibitor Sales Trend



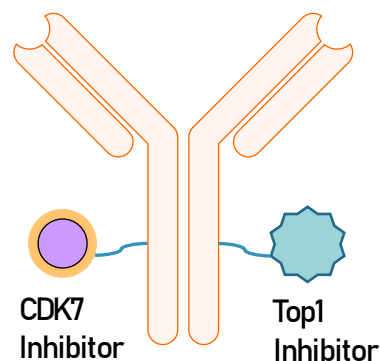
ADC Platform Roadmap

A scalable ADC platform extending from current standards to next-generation innovation

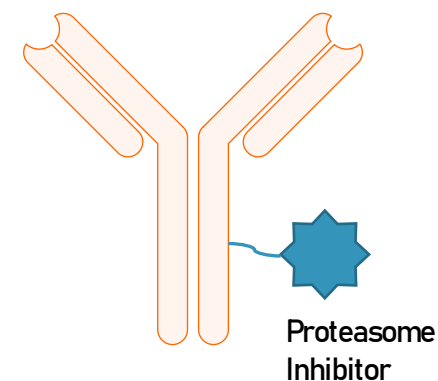
Q901 + ADC
combination



Dual payload
ADC



Novel payload
ADC

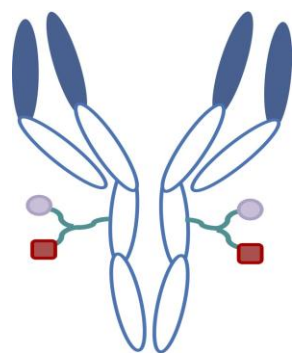


Development aligned with the future evolution of ADCs

QP101: A First-in-Class Dual-Payload ADC

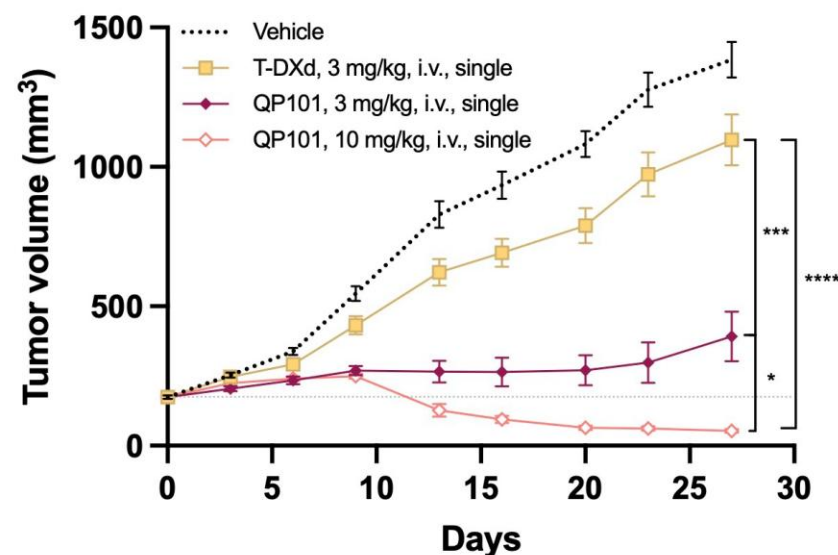
Overcoming the limitations of conventional Top1i ADCs

QP101

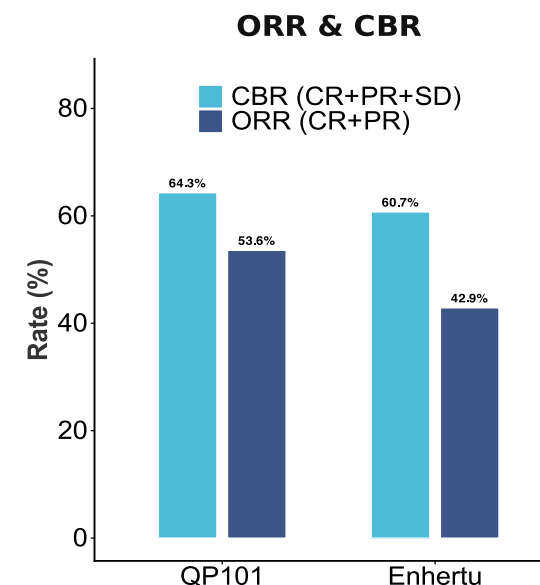


Dual-payload ADC
combining CDK7 inhibition
with TOP1i mediated
cytotoxicity

Efficacy of QP101 in Enhertu-resistant models



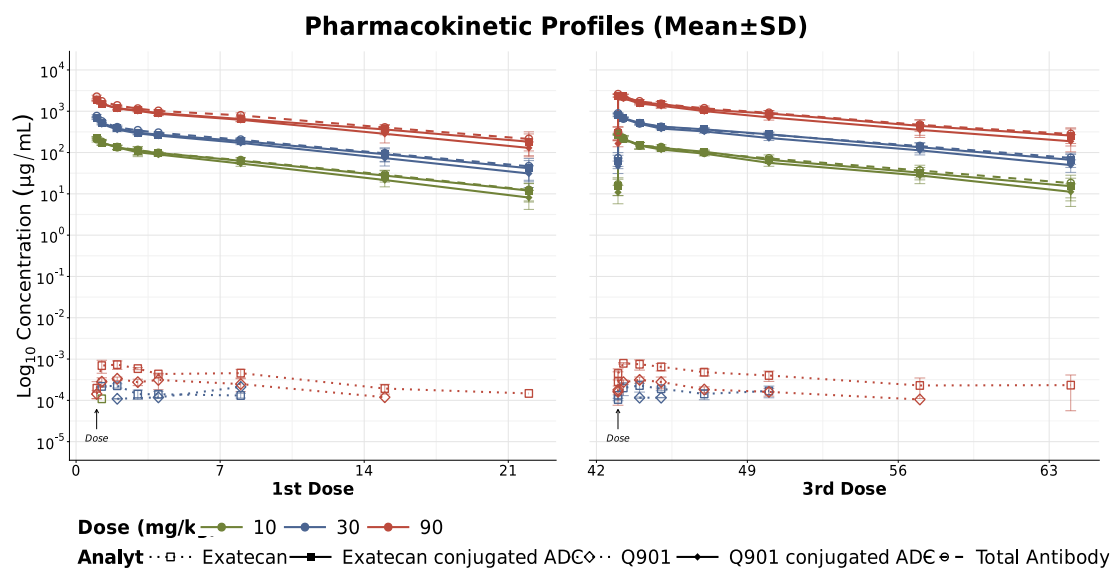
Response rate in patient-derived HER2+ models



ORR : QP101 (53.6%) vs. Enhertu (42.9%)

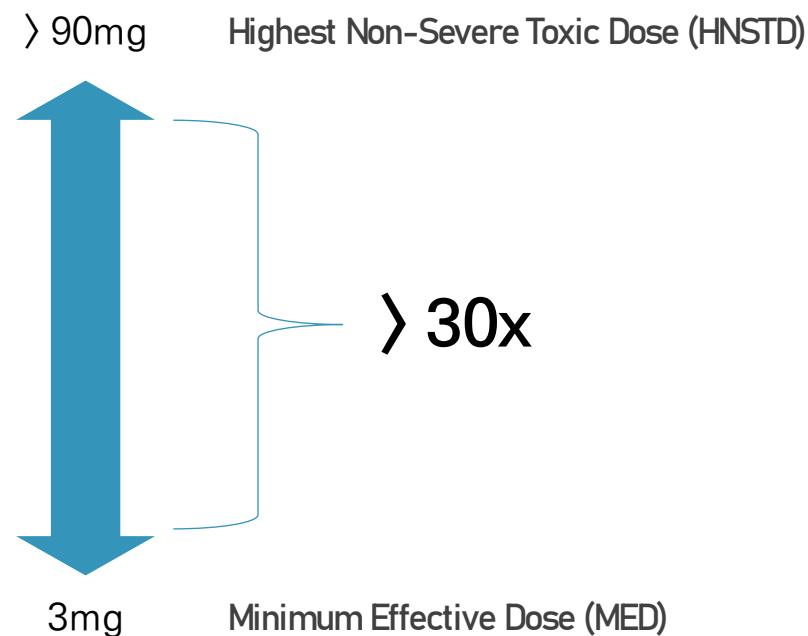
QP101: Differentiated Safety vs. Conventional ADCs

NHP Toxicology Study: Low Systemic Exposure to Free Payloads and a >30-Fold Therapeutic Index



- ✓ Dose-proportional systemic exposure across escalating dose levels
- ✓ Low systemic exposure to free payloads
- ✓ Provides the basis for QP101's favorable safety profile

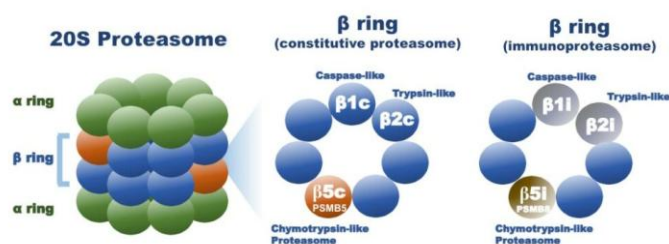
Wide Therapeutic Index (>30×)



Next-Generation PI-ADC with a Novel Mechanism of Action

Fully overcomes resistance to conventional ADCs through a mechanism independent of existing payload classes

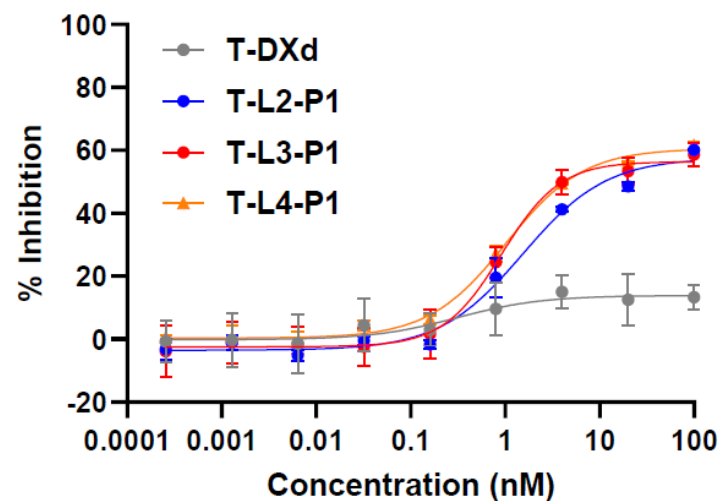
PI ADC platform



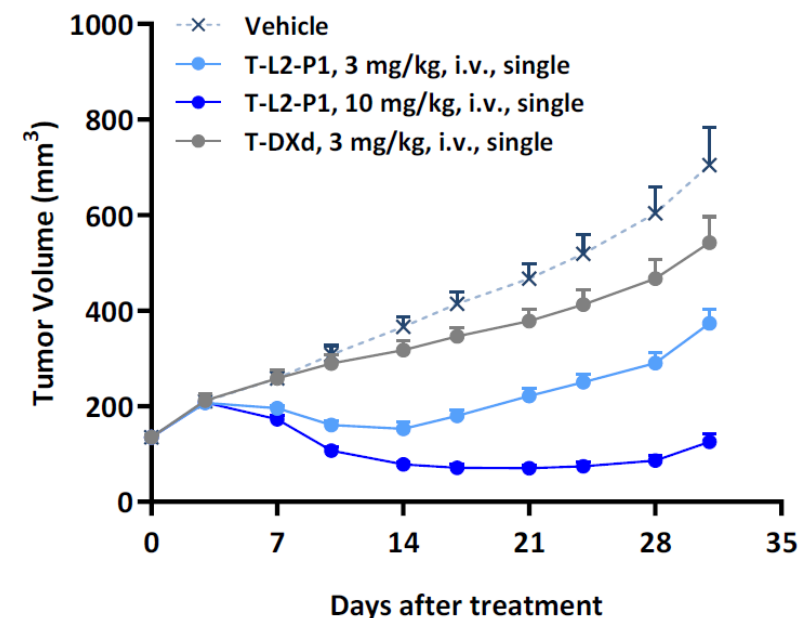
Selective targeting of constitutive and immunoproteasomes

- Reduced toxicity compared with existing inhibitors
- Broad therapeutic potential across oncology and autoimmune diseases

PI-ADC Activity in ADC-Resistant Model (ABCG2 drug efflux pump-overexpressing cancer cell model) *in vitro*

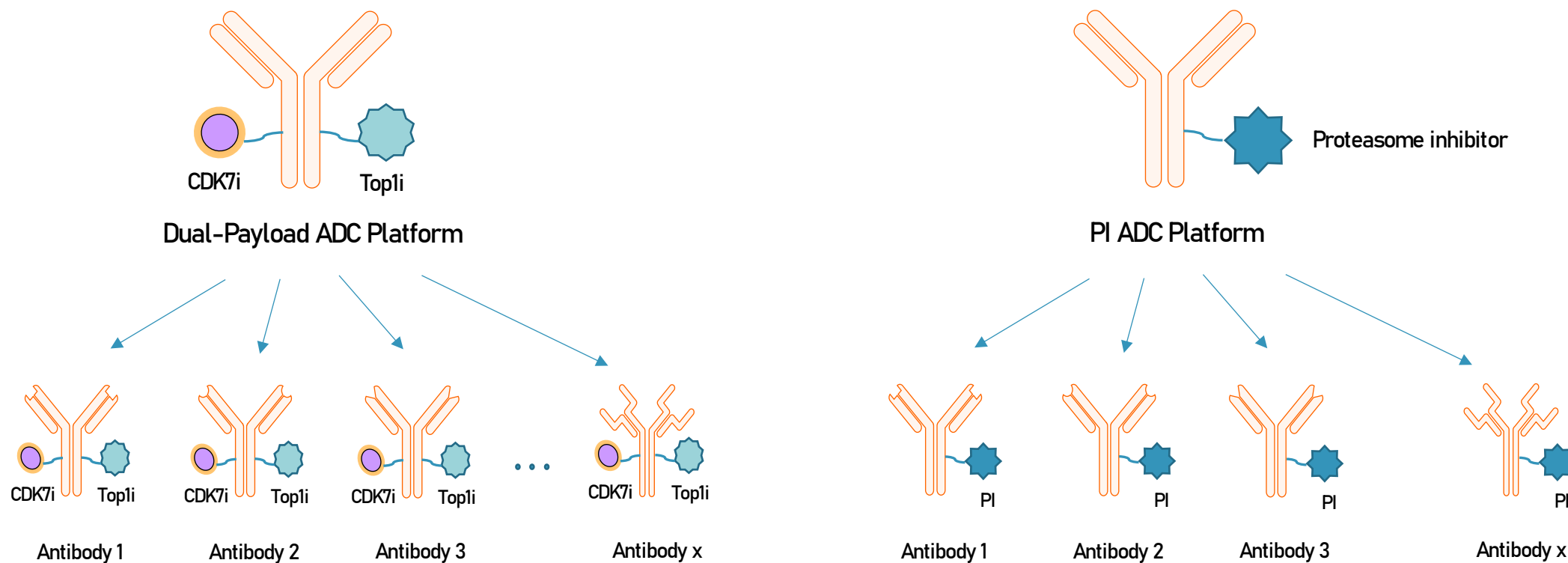


Superior PI-ADC in ADC-Resistant Model *in vivo*



A New Paradigm in ADCs – Modular Platform Architecture

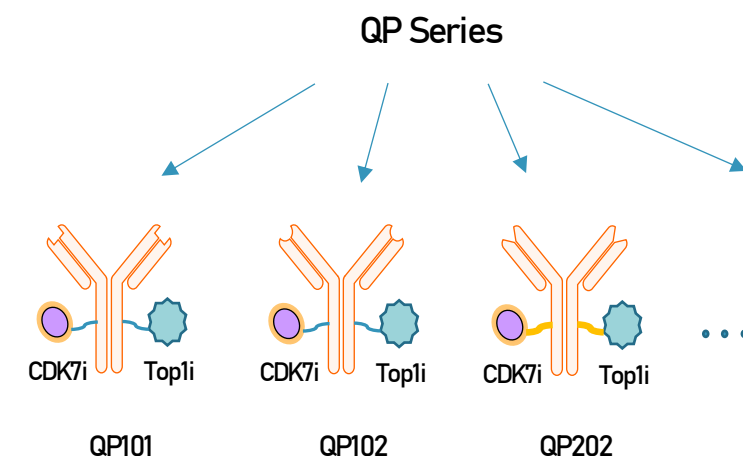
Modular ADC platforms enabling broad target expansion across multiple antibodies



Dual payload ADC Business Model

Proprietary QP-series pipeline development + platform licensing to external partners

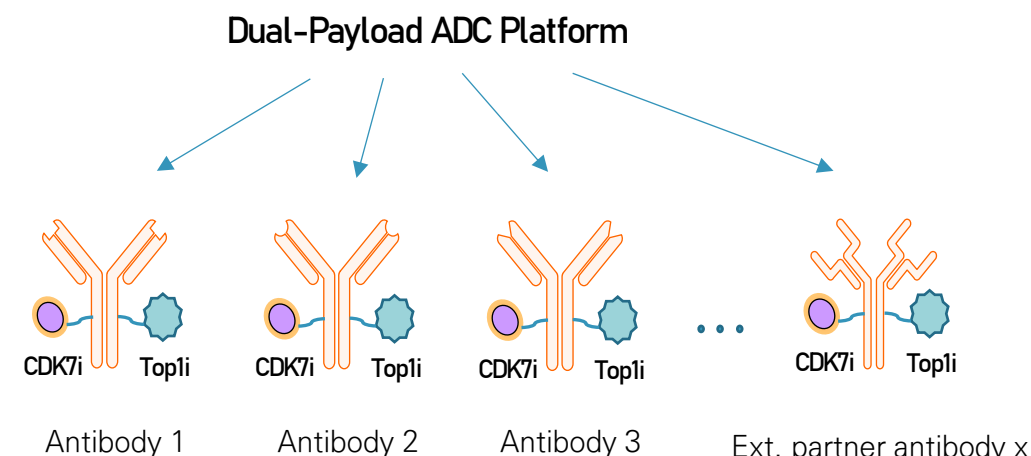
Maximizing Value Through Proprietary Pipeline Development



Asset Licensing

Potential for major out-licensing deals by individual asset

Multi-Partner Licensing Through Dual-Payload ADC Platform

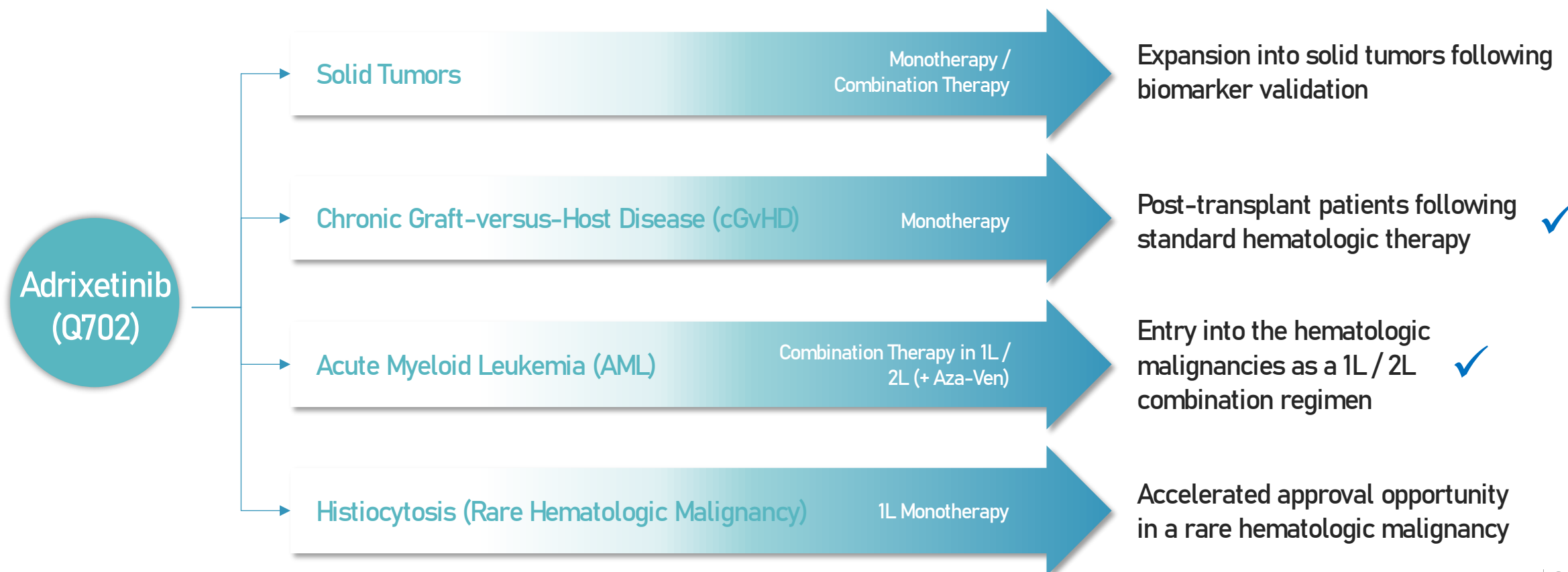


Platform Licensing

Repeatable licensing model across multiple partner antibodies

Adrixetinib (Q702) – Expanding Value across Multiple Indications

Prioritizing hematologic malignancies for rapid market entry and indication expansion



Telacebec – Regulatory Progress and PRV Monetization

Buruli ulcer registrational study ongoing → Eligible to receive a Tropical Disease PRV upon FDA approval

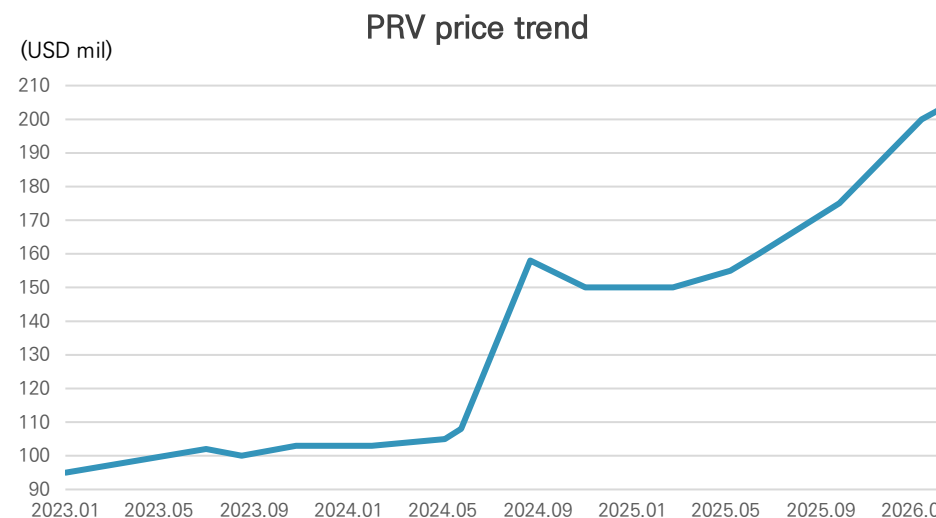
- A registrational study in Buruli ulcer is ongoing, with potential receipt of an FDA Priority Review Voucher (PRV) upon approval
- PRV prices have continued to rise amid limited supply and sustained demand (US\$215M in 3Q 2026)
- The Tropical Disease PRV is currently the only PRV program without a sunset provision

Telacebec development status

- ✓ First-in-class cytochrome bc1 inhibitor
- ✓ In development for tuberculosis, Buruli ulcer, and leprosy
- ✓ Global development led by TB Alliance

TREAT-BU Study

- ✓ Registrational study ongoing
- ✓ Rapid wound healing and high cure rates observed
- ✓ No recurrences reported among treated patients



It's Showtime

When science translates into value

- Historically, ADC development has focused on payload delivery technologies such as linkers, antibodies, and formulation. However, the payload itself ultimately determines efficacy and safety.
→ Qurient is positioned to lead the next wave of payload innovation
- Value realization from clinically validated pipelines through licensing, M&A (QLi5), and royalty income
→ Creates a self-sustaining cycle to fund the development of follow-on pipelines
- Potential Telacebec PRV value of approximately KRW 300 billion
→ Provides meaningful source of non-dilutive value and financial stability